



EMERGENCY MEDICAL DISPATCH PROTOCOLS



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PREFACE

These Emergency Medical Dispatch (EMD) Protocols are intended for use by trained and competent professionals as a guide to evidence-based best practices for out-of-hospital emergency care in developing health systems. They are not a substitute for training and should only be used by those specifically trained on the procedures documented within. This document is not intended to be a training tool or textbook. This document is reviewed and updated every two years, whilst open access with no copyright restrictions. If used, it is the user's responsibility to ensure that these protocols are the most up-to-date. Every effort has been made to ensure this document is free from errors. If one is found, please get in touch with the author, who will provide an update once it is released.

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OPERATIONAL DEFINITIONS

Ambulance station: A designated area or location where ambulances are stationed. An Emergency Medical Services System has many ambulance stations, but only one Emergency Medical Dispatch. Ambulance stations are selected and mapped using different criteria, which include population size, distance to a health facility, exposure and frequencies of emergencies or accident incidents, road jamming and type of road.

Dispatch Pre-Arrival Instruction (DPAI): Medically approved and scripted instructions to deliver necessary aid to the patient before EMS arrival.

Emergency Medical Care: Medical care given by EMS professionals at the scene and in ambulances.

Emergency Medical Service Providers (EMS Providers): Healthcare professionals working at the scene and in ambulances could be Emergency Medical Technicians (EMTs) or Paramedics. EMT could be at a basic or intermediate level, according to the training level or period.

Emergency Medical Service System (EMS System): A network of resources to deliver emergency care for the community outside of healthcare facilities and in healthcare facilities, and includes prevention of emergency conditions and rehabilitation of emergency-affected patients.

Emergency conditions: Any medical emergency conditions of all ages, genders, and events (medical, injury, obstetrics, gynaecology, and children)

Emergency Medical Dispatch (EMD) /Ambulance Dispatch/Centre: An organisation that serves as a mediator between the community and EMS providers for further detailed integration about the medical illness characteristics to allocate the proper level of responses. The centre is equipped with electronic and computerised communication devices, protocols, professionals who can handle and prioritise the response and a GIS that displays the ambulance station locations.

Emergency Medical Dispatcher: A trained professional who links the caller requesting emergency medical assistance and the emergency medical service (EMS). Emergency Medical Dispatchers are organisers of the EMS system to respond to a reported and subsequently verified medical emergency.

Emergency Medical Dispatch Protocol: A written structure of actions for evaluating, responding to, and providing care to emergency patients.

Medical oversight: A qualified emergency healthcare provider who provides emergency medical consultation for dispatchers and ambulance crews. They are responsible for the ambulance crew and dispatchers, as well as service evaluation and quality of care improvement, and can act directly or indirectly.



1.0

1.0: INTRODUCTION

Emergency medical dispatch (EMD) is a system in which trained dispatchers provide medical assistance and emergency medical services to callers in need over the phone. EMD dispatchers are responsible for gathering information from callers, assessing the situation, and dispatching the appropriate emergency medical services (EMS) resources to the scene. EMD is essential to the emergency medical services system, providing a critical link between a caller and EMS providers. EMD dispatchers ensure appropriate care is delivered quickly and efficiently to those in need.

Emergency Medical Dispatch (EMD) practice is based on consistently using medically approved dispatch protocols. These protocols are standardised procedures used by emergency medical dispatchers to evaluate the nature and severity of the emergency, prioritise calls based on the level of urgency, and provide pre-arrival instructions to callers to help stabilise the patient until EMS arrives. These protocols are based on the best practices and guidelines developed by national and international emergency medical organisations and are continuously updated to reflect current evidence-based practices.

1.1: Principles of Emergency Medical Dispatch

Emergency Medical Dispatch (EMD) is a critical component of the emergency response system. The following are some of the principles of EMD:

- a) **Stay Calm and Collected:** Emergency dispatchers should always remain calm and composed, even when dealing with the most stressful and chaotic situations. They must reassure callers that help is on the way and provide instructions to help manage the situation until responders arrive.
- b) **Gather Information:** Dispatchers must gather as much information as possible from callers to provide the most accurate assessment of the situation. This includes obtaining the location of the incident, the nature of the emergency, and any medical information about the patient.

- c) **Prioritise the Emergency:** Dispatchers must use protocols properly and have good judgment to prioritise emergencies based on the severity level and the potential threat to life or property. They must quickly assess the situation and provide the appropriate level of response. The nearest available ambulance must be tasked to the highest priority incident. The Dispatcher may preserve the availability of ambulances by a stand-in-line mechanism for priority incidents until sufficient resources are available.
- d) **Provide Pre-Arrival Instructions:** Dispatchers must provide pre-arrival instructions to callers, which may include basic first aid, CPR, or other life-saving techniques. This can help save lives and minimise the risk of further injury or damage until responders arrive.
- e) **Maintain Communication:** Dispatchers must maintain communication with both the caller and emergency responders throughout the incident. This helps ensure that the situation is managed effectively and that any changes to the situation are communicated to responders in real time. When the response is going to be delayed, Dispatchers shall inform the caller of the estimated time of arrival and follow the patient's condition using relevant questioning of the caretaker and advise care to be given by bystanders or use other alternatives.
- f) **Continuous Training:** Dispatchers must receive continuous training to stay up-to-date on the latest emergency response protocols and technologies. This helps to ensure they can effectively manage any emergency.

1.2: Emergency Medical Dispatch Protocols

Emergency Medical Dispatch (EMD) protocols are a set of guidelines and procedures that dictate the actions of emergency medical dispatchers during emergencies. The protocols provide a standardised approach to dispatching emergency medical services (EMS) and ensure patients receive appropriate and timely care. The protocols include;

- a) Systematised caller-interrogation questions that are specific to the chief complaint
- b) Systematised pre-arrival instructions
- c) Protocols that determine ambulance response mode according to the chief complaint severity
- d) Referenced information for dispatcher use.

Emergency Medical Dispatch (EMD) protocols have several benefits in emergencies. These protocols provide a standardised and structured approach to handling emergency calls, which can help to improve the quality and efficiency of emergency medical services. They enable the EMD to avoid making mistakes in medical emergencies and incorrect dispatching decisions. When dispatchers fail to use medical dispatch protocols, they may be prone to assess the situation based on inadequate information, fail to identify the patient's chief complaint, and, therefore, may provide insufficient response or advice.

Additional benefits include:

- Helps to answer all the essential questions that will be asked during a medical emergency
- Helps the Dispatcher determine the level and mode of the responses
- Helps to minimise long conversations and speed up the response
- Helps to use specifically defined determinant descriptors
- Helps to make the service persistent and minimises individual variation
- Defines incident characteristics and facilitates decisions
- Protocols can be used for quality assurance (QA) and quality indicators (QI)

- All callers can expect the same level of service as per the set standards
- Reduces inconsistency, which lessens the risk for errors, thus reducing liability biases that can cause a dispatcher who isn't following a set structure to "lead" a caller along an inappropriate information-gathering pathway
- Protocols help analyse and classify symptoms.
- Protocols organise vast amounts of information for consideration by the decision-maker.
- Protocols show the interrelationship of various data, forcing consideration of all possible decision choices and safeguarding against stereotyping.
- Protocols can help to reconstruct the decision-making process.

Emergency Medical Dispatch (EMD) protocols must be medically approved, uniform throughout each EMS jurisdiction, use standard response classification codes, and be followed consistently and non-arbitrarily by all EMDs. The specific protocols emergency medical dispatchers follow may vary depending on the jurisdiction and level of care provided. However, all protocols must aim to provide emergency patients with timely and effective medical care. Regular training and review of protocols are essential to ensure that emergency medical dispatchers are equipped to handle any emergency.

1.3: Emergency Medical Dispatchers

Emergency Medical Dispatchers must have appropriate training on program administration, periodical supervision, and medical direction so they can accurately collect information from the caller, select an appropriate response method, provide pertinent information to responders and give appropriate aid and direction to caretakers of the patient or caller. To do this, there should also be a written and medically approved emergency medical dispatch protocol that helps their decisions concerning EMS responses in a safe, reproducible, and non-arbitrary manner.

1.4: Role and Responsibilities of Emergency Medical Dispatchers

The role and responsibilities of an Emergency Medical Dispatcher (EMD) are critical in providing emergency medical care and assistance to patients in need. Here are some of the primary responsibilities of Emergency Medical Dispatchers:

- a) **Answering emergency calls:** Answering emergency calls and gathering essential information from the caller about the nature of the emergency and the patient's condition.
- b) **Prioritising calls:** Prioritising emergency calls based on the severity of the situation, the patient's condition, and the resources available.
- c) **Dispatching emergency medical services:** Dispatching the appropriate level of emergency medical services, such as ambulances, helicopters, or first responders, based on the priority level of the call.
- d) **Providing pre-arrival instructions:** Providing pre-arrival instructions to the caller, such as performing CPR or controlling bleeding to stabilise the patient's condition before emergency medical services arrive.
- e) **Coordination with other emergency services and incident management:** Dispatchers work closely with other emergency services, such as police, fire departments, and hospitals, to ensure a coordinated response and the best possible outcome for the patient. The Dispatcher may also coordinate with other emergency services, such as police or fire departments, to manage the incident and ensure the safety of all involved.
- f) **Monitor the situation:** Monitoring the situation and updating the responding EMS personnel as necessary to ensure they have the most up-to-date information and are prepared to provide the appropriate level of care.
- g) **Documentation and record-keeping:** Maintaining accurate and complete records of all emergency calls and dispatches, including patient information, the nature of the emergency, and the response provided.

Emergency Medical Dispatchers must also;

- Have contingency plans and procedures to accommodate any system failures.
- Precisely analyse the information provided by the caller to allow appropriate resources to be prioritised and allocated at the earliest possible opportunity.
- Based on the caller's information, ascertain scene safety and inform the responding crew.
- Be professional and patient while responding to angry and distressed callers.
- Provide telephone advice and guidance to callers regarding any emergency medical conditions using the Emergency Medical Priority Dispatch System.
- Collaborate with health professionals working in health institutions
- Work closely with other emergency service agencies, fire, police, private and NGO ambulances
- Have a precise knowledge of how and when to involve medical oversight on a variety of issues, such as incidents that have the potential to become large-scale, including all Chemical, Biological, Radiological, and Nuclear (CBRN) incidents.
- Use the appropriate trust reporting mechanisms for any adverse incidents
- Share best practices with colleagues
- Make suggestions for improvements and developments to systems and processes to the appropriate managers.
- Maintain own Continuing Professional Development
- Maintain principles of patient confidentiality
- Adhere to the Emergency Medical Dispatch Code of Ethics
- Attend all mandatory training required to maintain Emergency Medical Dispatch registration



2.0

2.0: CALLER INTERROGATION

Systematised interrogation is an essential component of a comprehensive medical dispatch protocol, even for those systems not prioritising between Advanced Life Support (ALS) and Basic Life Support (BLS) calls. The Dispatcher asks specific questions to gather information about the nature of the emergency and the patient's condition.

The goal of caller interrogation is to obtain accurate and comprehensive information that enables the Dispatcher to determine the appropriate level of response and provide pre-arrival instructions to the caller. The following questions are asked during the caller interrogation:

- a) **Location:** The Dispatcher will ask the caller for the exact location of the emergency, including the street address, nearest cross street, and any landmarks that may help identify the location.
- b) **Nature of the emergency:** The Dispatcher will ask the caller to describe the nature of the emergency, including any injuries, symptoms, or medical conditions that the patient is experiencing.
- c) **Patient information:** The Dispatcher will ask the caller for information about the patient, such as their age, gender, and level of consciousness.
- d) **Identify:** Major haemorrhage or circulatory collapse, altered conscious status, airway patency and respiration quality.
- e) **SAMPLE History:** The Dispatcher may ask the caller about;
 - Signs and Symptoms
 - Allergies
 - Medications the patient is on
 - Past medical history; Pregnancy
 - Last meal and medication intake
 - Events preceding

The Dispatcher uses the information gathered during the caller interrogation to prioritise the call, allocate appropriate resources, and provide pre-arrival instructions to the caller. Emergency medical dispatchers need to be skilled in asking the right questions and obtaining accurate information quickly, as this can help save lives and ensure the best possible outcomes for patients in emergencies.



3.0: PRIORITY DETERMINATION

Priority determination is a critical component of Emergency Medical Dispatch (EMD) protocols, which involves the Dispatcher prioritising the call based on the severity of the emergency and the patient's condition. The goal of priority determination is to ensure that the appropriate level of response is dispatched to the scene promptly, thereby helping to save lives and improve patient outcomes.

Emergency medical dispatchers use Emergency Medical Dispatch (EMD) codes to categorise and prioritise emergency calls based on the type and severity of the situation. EMD codes are standardised and help ensure that the appropriate level of response is dispatched to the emergency. Here are some standard EMD codes:

- a) **Priority 1 (Life-threatening):** This code is used for the most serious, life-threatening emergencies, such as cardiac arrest, severe bleeding, or respiratory distress. A Priority 1 response typically involves sending an ALS ambulance.
- b) **Priority 2 (Emergency):** This code is used for serious but not immediately life-threatening emergencies, such as chest pain or a broken bone. A Priority 2 response typically involves sending an ILS Ambulance or possibly an ALS Ambulance.
- c) **Priority 3 (Minor injuries/illnesses):** This code is used for non-emergency medical situations, such as minor injuries or illnesses. A Priority 3 response typically involves sending A BLS Ambulance or non-emergency medical transport.
- d) **Priority 4 (Backup response):** This code is used for non-medical emergencies like car accidents or fires. A Priority 4 response typically involves sending the appropriate emergency services, such as the police or fire department.
- e) **Priority 5 (Referral/ Medical Transport):** This code is used for information requests, such as a request for a referral to a medical facility or a non-emergency medical transport.

The use of EMD codes helps emergency medical dispatchers quickly and accurately categorise emergency calls and dispatch the appropriate level of response to the scene.



4.0

4.0: DISPATCHING EMERGENCY MEDICAL SERVICES (EMS)

The EMD is crucial in dispatching EMS and ensuring a coordinated and effective emergency response. Their ability to gather important information, assign priority levels, and dispatch the appropriate resources can help save lives and improve patient outcomes in emergencies. To allocate resources effectively and efficiently, EMDs need to take the following steps;

- a) **Gather information:** The first step in resource allocation is to gather as much information as possible about the emergency. Ask questions to understand the nature and severity of the situation, the location of the patient, and any other relevant details.
- b) **Prioritise the response:** Prioritise the response based on the information you gather. Assign the appropriate level of response based on the severity of the emergency. This may involve dispatching an ambulance, fire truck, or other emergency vehicles.
- c) **Coordinate with other agencies:** In some cases, you may need to coordinate with other emergency services agencies, such as law enforcement or fire departments, to ensure the response is appropriate and effective.
- d) **Communicate clearly:** Effective communication is critical in emergencies. Ensure you communicate clearly and effectively with all parties involved, including emergency responders, medical personnel, and the caller.
- e) **Monitor the situation:** Monitor the situation closely once the response is underway. Stay in communication with all parties involved and adjust the response as needed.
- f) **Document the response:** Document the response in detail after the emergency. This information can be used to improve the response process in the future.

4.1: AMBULANCE RESPONSE MODES

Ambulance response modes are the different response levels used by ambulance services to prioritise emergency calls and determine the level of care required for the patient. The response modes are based on the severity of the medical emergency and the level of resources needed to respond effectively. Here are the four standard ambulance response modes:

- a) **Code 1:** Also known as **Emergency Mode** or **Code Red**, this mode is used for the most urgent and life-threatening emergencies, such as cardiac arrest or severe trauma. In this mode, the ambulance will use its lights and sirens to respond to the scene and provide rapid, immediate medical care.
- b) **Code 2:** Also known as **Urgent Mode** or **Code Yellow**, this mode is used for urgent but non-life-threatening emergencies, such as chest pain or respiratory distress. In this mode, the ambulance will use lights and sirens to respond quickly to the scene, but may not need to transport the patient to the hospital with lights and sirens activated.
- c) **Code 3:** Also known as **Non-Emergency Mode** or **Code Green**, this mode is used for non-life-threatening medical situations, such as transportation to a medical appointment or a non-urgent medical condition. In this mode, the ambulance will respond without lights and sirens and transport the patient to the hospital without urgency.
- d) **Standby Mode:** This mode is used for events or situations where an ambulance needs to be on standby, such as a large gathering or public event. In this mode, the ambulance will be parked at a designated location and ready to respond if needed.

Different ambulance response modes allow ambulance services to prioritise emergency calls and allocate resources efficiently. The specific response mode for each call is based on the level of urgency and the resources needed to provide adequate care.





5.0

5.0: DISPATCH PRE-ARRIVAL INSTRUCTIONS (DPAI)

Dispatch Pre-arrival instructions are medically approved and scripted instructions to deliver necessary aid to the patient before EMS arrival. The instructions aim to provide essential information to help the person manage the patient's condition and prepare them for further medical assistance upon arrival. Dispatch pre-arrival instructions may include:

- a) **Obtain the caller's contact information:** Obtain the caller's contact information in case the emergency responders need to follow up or clarify any details.
- b) **Give specific directions:** Get detailed instructions on how to get to the emergency location, including any particular landmarks, building numbers, or entrances.
- c) **Advise the caller to stay calm:** Encourage them to remain calm and reassure them that help is coming.
- d) **Provide safety instructions:** If there are any potential hazards or risks at the location, such as fire or gas leaks, advise the caller to take precautions and move to a safe area.
- e) **Assess the patient:** Ask the caller to assess the patient's condition and provide any relevant information. This will help the emergency responders to better prepare for the situation.
- f) **Provide first aid instructions:** If the caller has any medical training, provide them with instructions on how to administer basic first aid until the emergency responders arrive.
- g) **Request additional resources:** If necessary, request additional resources, such as police or fire department assistance, and provide details on the nature of the emergency.



6.0

6.0: EMERGENCY MEDICAL RESPONSE PROTOCOLS

Emergency Medical Response (EMR) protocols are guidelines and procedures that dictate the actions of emergency medical service (EMS) personnel during emergencies. The protocols provide a standardised approach to EMS care and ensure that patients receive appropriate and effective treatment promptly. Here are some standard emergency medical response protocols:

- **Scene assessment:** Upon arrival at the scene, EMS personnel will assess the situation to determine the nature of the emergency, any hazards or risks present, and the number of patients.
- **Patient assessment:** EMS personnel will assess the patient's condition and vital signs, such as heart rate, blood pressure, and breathing rate, to determine the appropriate course of treatment.
- **Airway management:** If the patient has difficulty breathing or maintaining their airway, EMS personnel will provide interventions such as oxygen administration or basic or advanced airway management.
- **Cardiopulmonary resuscitation (CPR):** If the patient is unresponsive and not breathing, EMS personnel will perform CPR to restore circulation and breathing.
- **Medication administration:** EMS personnel may administer medications, such as pain relievers or anti-anxiety medications, as needed to manage the patient's symptoms.
- **Patient transport:** EMS personnel will transport the patient to the appropriate medical facility based on the patient's condition and the hospital's capabilities.
- **Communications:** EMS personnel will communicate with other healthcare providers, such as hospital staff or other EMS personnel, to accurately report the patient's condition and treatment.

The specific protocols EMS personnel follow may vary depending on the jurisdiction and the level of care provided. However, all protocols aim to provide emergency patients with timely and effective medical care. Regular training and review of protocols are essential to ensure that EMS personnel are equipped to handle any emergency.



7.0

7.0: COORDINATION WITH OTHER EMERGENCY SERVICES AND INCIDENT MANAGEMENT

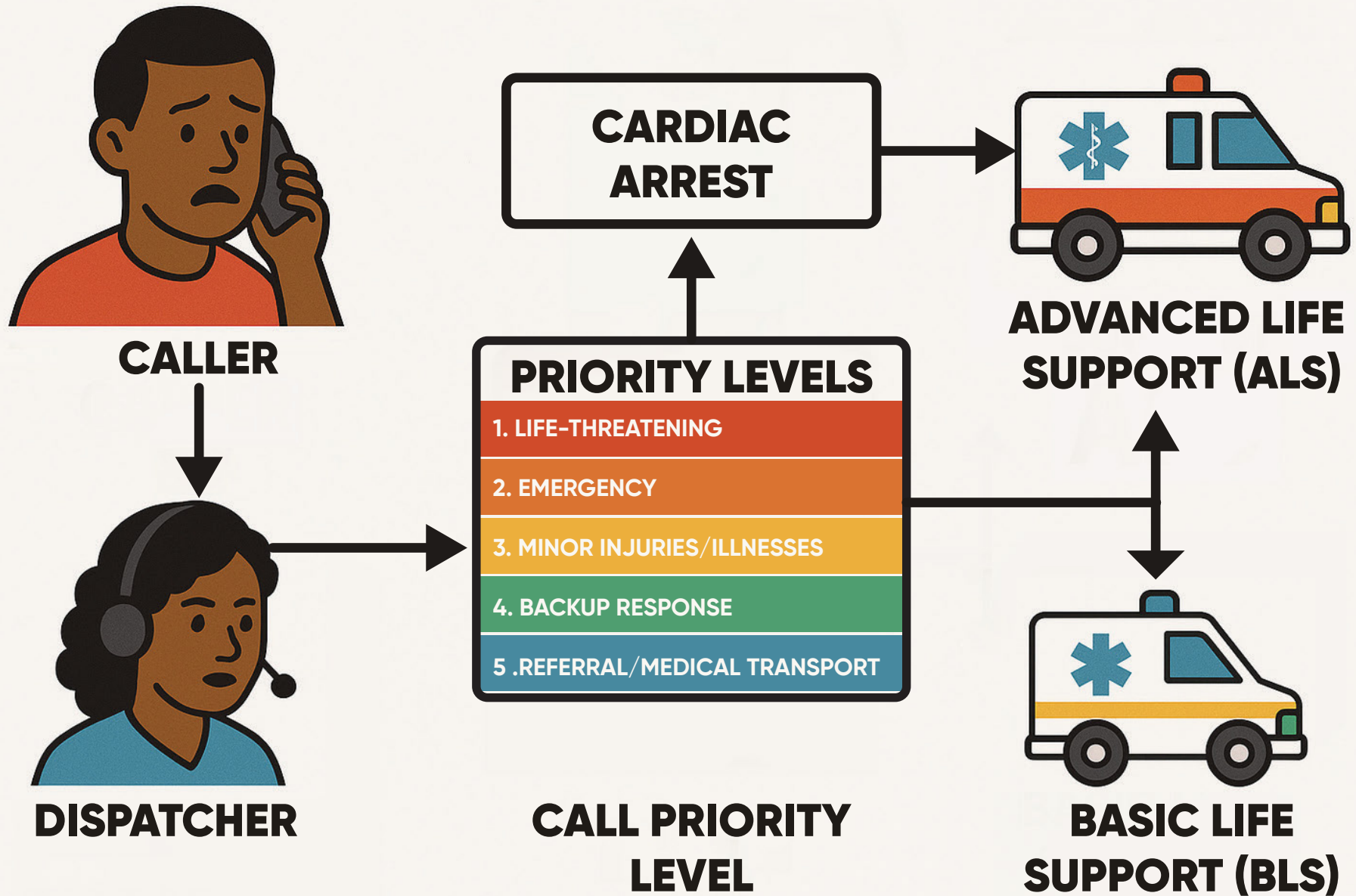
Emergency Medical Dispatch (EMD) plays a critical role in coordinating with other emergency services to provide an effective and efficient emergency response. Effective coordination between emergency services is essential to ensure that resources are allocated appropriately and that the response is coordinated and effective. This includes;

- a) **Dispatching emergency services:** The primary role of EMD is to dispatch emergency services to the scene of an emergency. This may include dispatching ambulance services, fire departments, or law enforcement agencies, depending on the nature and severity of the emergency.
- b) **Providing information to other emergency services:** EMDs gather information about the emergency and provide it to other emergency services to ensure they are fully informed about the situation.
- c) **Coordinating resources:** EMDs work with other emergency services to coordinate resources and ensure that the appropriate level of resources is dispatched to the scene. This may include coordinating the use of specialised equipment or personnel.
- d) **Communicating with emergency services:** EMDs maintain communication with emergency services throughout the emergency response process. This ensures that all parties are fully informed about the situation and can make informed decisions.
- e) **Providing support to other emergency services:** EMDs may support other emergency services during the response process. This may include providing medical instructions or advice, coordinating with other emergency services, or providing different forms of support as needed. Dispatchers may also flag potentially distressing incidents to trigger mental health and psychosocial support for the activated responders. e.g. multiple fatalities, paediatric trauma, suicide/attempt, etc.

Incident management involves a coordinated and systematic approach to managing emergencies, including responding to and recovering from the incident. The role of EMD in incident management is critical, particularly in the early stages of an incident, to ensure a coordinated and effective response to emergencies. The EMD plays a crucial role in;

- a) **Early detection and reporting:** EMDs are often the first to detect and report an incident to the appropriate emergency services. Early detection and reporting can help emergency services respond more quickly and effectively.

- b) **Situational awareness:** EMDs are responsible for gathering and assessing information about the incident. This information can be used to develop a complete picture of the incident and inform the response.
- c) **Coordination with other emergency services:** EMDs work closely with other emergency services, including police and fire departments, to coordinate the response to the incident. Effective coordination is crucial to ensure that resources are allocated efficiently and that the response is well-coordinated and effective.
- d) **Support for emergency responders:** EMDs support emergency responders in the field by providing medical instructions, coordinating resources, and other support forms as needed.
- e) **Monitoring and tracking:** EMDs are responsible for monitoring and tracking the incident and providing regular updates to emergency services and other stakeholders. This helps to ensure that all parties are fully informed about the situation and can make informed decisions.





8.0

8.0: COMMUNICATION

Effective communication is critical in EMD as it enables EMDs to gather accurate and complete information, provide appropriate medical instructions, and coordinate the response with other emergency services. Effective communication is also essential to ensure that the appropriate level of resources is dispatched to the scene and that the response is coordinated and effective. Some key aspects of communication in EMD include:

- **Active listening:** EMDs must be active listeners, carefully listening to callers to ensure that they gather accurate and complete information about the emergency. Active listening involves paying attention to the caller's tone of voice, inflexion, and any other clues that may help the EMD better understand the situation.
- **Clarity and brevity:** EMDs must communicate clearly and concisely to ensure that information is conveyed accurately and efficiently. This is particularly important in emergencies where time is of the essence.
- **Empathy:** EMDs must also communicate with empathy, demonstrating compassion and understanding towards callers in distress or experiencing a medical emergency.
- **Medical terminology:** EMDs must be fluent in medical terminology to ensure that they can accurately describe medical conditions and provide appropriate medical instructions to callers. Dispatchers should also be able to tailor their language to define actions to the layperson/bystander.
- **Coordination with other emergency services:** EMDs must communicate effectively with other emergency services, including police and fire departments, to coordinate the response and ensure that resources are allocated appropriately.



9.0

9.0: EMD PROTOCOL GUIDE CARDS

9.0.1: EMD Categories and Types

Category	Types	EMD Protocol Card Name
Trauma	Animal Bites	9.4 Animal Bites
	Assault (Domestic, Sexual)	9.5 Assault (Domestic, Sexual)
	Bleeding	9.7 Bleeding
	Burns	9.9 Burns
	Cardiac Arrest	9.10 Cardiac Arrest
	Drowning	9.15 Drowning
	Electrical Injury	9.16 Electrical Injury
	Epistaxis	9.17 Epistaxis
	Explosion	9.18 Explosion
	Eye Injuries	9.19 Eye Injuries
	Falls	9.20 Falls
	Fractures	9.22 Fractures
	Hanging / Strangulation	9.23 Hanging / Strangulation
	Industrial Accidents	9.26 Industrial Accidents
	Road Traffic Crash	9.33 Road Traffic Crash
	Sprains	9.35 Sprains
	Stabbings / Gunshots / Assault	9.36 Stabbings / Gunshots / Assault
	Traumatic Injury	9.38 Traumatic Injury

Category	Types	EMD Protocol Card Name
Medical	Abdominal Pain	9.1 Abdominal Pain
	Allergies/Stings	9.2 Allergies/Stings
	Altered Mental State	9.3 Altered Mental State
	Back Pain	9.6 Back Pain
	Bleeding	9.7 Bleeding
	Breathing Problems	9.8 Breathing Problems
	Cardiac Arrest	9.10 Cardiac Arrest
	Chest Pain / Heart Problems	9.11 Chest Pain / Heart Problems
	Choking	9.12 Choking
	Diabetic Complications	9.14 Diabetic Complications
	Febrile Child	9.21 Febrile Child
	Headache	9.24 Headache
	Heat / Cold Exposure	9.25 Heat / Cold Exposure
	Infectious Disease	9.27 Infectious Disease
	Persistent Vomiting and Diarrhoea in Children	9.28 Persistent Vomiting and Diarrhoea in Children
	Poisoning – Ingestions / Overdose	9.29 Poisoning – Ingestions / Overdose
	Poisoning - Inhalation / Carbon Monoxide	9.30 Poisoning - Inhalation / Carbon Monoxide
	Psychiatric / Behavioural Problems	9.32 Psychiatric / Behavioural Problems
	Seizures / Convulsions	9.34 Seizures / Convulsions
	Stroke/CVA	9.37 Stroke/CVA
	Unknown illness	9.39 Unknown illness
	Unresponsive Patient	9.41 Unresponsive Patient
	Weakness of Extremities	9.43 Weakness of Extremities
Pregnancy	Cord Prolapse	9.13 Cord Prolapse
	Pregnancy / Childbirth	9.31 Pregnancy / Childbirth
	Vaginal Bleeding	9.42 Vaginal Bleeding

9.0.2: GENERAL PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS

The following pre-arrival instructions/interventions should be applied to all calls;

- a) Always consider bystander and patient safety.
- b) Always consider the potential criticality of the following groups: pregnant women, paediatric patients, geriatric patients, persons with disability or frailty, and persons with infectious diseases.
- c) Always ensure good patient positioning.
- d) Always continue to assist or monitor the patient until help arrives
- e) Always attempt to gather more details about the person's symptoms, medical history, allergies, or any medications they are taking.
- f) Always call back if the condition of the patient deteriorates or changes
- g) Always try to have someone meet the ambulance team

9.0.3: MASS CASUALTY/MAJOR INCIDENT

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. What is your exact location? 2. What is the nature of the incident? 3. Are there any immediate dangers to responders or bystanders? 4. How many people are injured or affected? 5. What types of injuries or medical conditions are the affected individuals experiencing? 6. Are there any individuals who are trapped or unable to move? 7. Are there any hazards or obstacles that could impede the response of emergency services? 8. Have any bystanders provided medical assistance or interventions? 9. Are any additional resources or personnel needed on site? 10. Can you provide any additional information that may assist the emergency services in their response?	Simultaneous ALS/BLS • Patients with a decreased level of consciousness • Accident with crushing or penetrating injuries to the head, neck, torso, thigh • Patients entrapped. Amputation other than fingers/toes • Patients paralysed • Patients with uncontrolled bleeding • Multiple extremity fractures. • Femur (thigh) fractures. BLS Dispatch • Amputation/entrapment of fingers/toes • Neck or back pains without critical symptoms • Patients with controlled bleeding • Patients with cuts, bumps, or bruises • Involved in an accident, no major complaints	1. General pre-arrival instructions/interventions 2. Assess the situation and ensure you and other bystanders are safe. If there are any immediate dangers, such as a fire, gas leak, or ongoing violence, move to a safe location. 3. If you have any medical training or experience, assist the injured individuals, e.g., applying pressure to wounds, performing CPR, or administering basic first aid. 4. If you are unable to provide medical assistance, try to monitor the injured individuals and provide comfort and reassurance. Stay with them until emergency services arrive. 5. Clear the path for emergency services vehicles to reach the scene, e.g., directing traffic or guiding responders to the location of the incident.

9.1: ABDOMINAL PAIN

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the pain due to an injury to the patient? 2. How does the patient feel sitting up? 3. Is the pain above or below the belly button? If the patient is female between 12 and 50 years: 4. Could she be pregnant? 5. Has there been vaginal bleeding? IF YES, how much? Has she said she felt dizzy? 6. Has the patient vomited? IF YES, what does the vomit look like? 7. Are the patient's bowel movements black and tarry? 8. Is the patient wearing a Medic Alert tag? IF YES, what does it say? 9. Does the patient have Addison's Disease, recent trauma or any other medical or surgical history? 10. Has the patient recently travelled outside of the state or country? IF YES, Where? (Check ALERTS)	Simultaneous ALS/BLS • Lower abdominal pain in a woman 12-50 years (if associated with dizziness, fainting, or heavy vaginal bleeding). • Vomiting blood (red/dark red) or coffee ground-like substance • Black tarry stool • Upper abdominal pain with a prior history of heart problems. • Abdominal pain with fainting or near fainting, patient over 50 years. • Pulsatile abdominal mass • Fainting/near fainting when sitting. (hypotension) • Pain with prior history of Addison's disease or adrenal insufficiency. <i>Symptoms of an Addison or "Adrenal" crisis include:</i> - Severe vomiting and diarrhoea - Dehydration - Low blood pressure - Loss of consciousness If not treated, an Addison crisis can be fatal. BLS Dispatch • Pain with vomiting. • Flank pain (Kidney stone). • Abdominal (non-traumatic). • Pain unspecified.	1. General pre-arrival instructions/interventions 2. Nothing to eat or drink. 3. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils.

9.2: ALLERGIES / STINGS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Does the patient have a history of a reaction to anything? IF YES, describe the reaction the patient had before. - Is the patient having difficulty swallowing or difficulty breathing, or both? - Is the patient complaining of itching, hives, or rash? - Are the symptoms getting worse? - Is the patient wearing a Medic Alert tag? IF YES, what does it say? - How does the patient act when they sit up?	Simultaneous ALS/BLS - Unconscious/not breathing normally. - Decreased level of consciousness. - Difficulty breathing. - Difficulty swallowing. - Cannot talk in complete sentences. - Swelling in the throat or on the face. - Fainting. - History of a severe reaction. - Itching or hives in multiple areas BLS Dispatch - Call delayed longer than 30 minutes with a history of reaction. - Concern about reaction, but no history. Reaction presents for a long time (hours), with no difficulty breathing. Itching or hives in one area	General pre-arrival instructions/interventions - Do you have an Epi-Pen or reaction kit? IF YES, have you used it as directed? If they have not used it, use it following the directions on the kit. - Brush the stinger off, if possible. Do not attempt to grasp the stinger. - Apply ice to the site of the sting. - Keep neck straight and remove pillows. - Watch the patient for signs of difficulty breathing (slow breathing) or cardiac arrest. Go to the appropriate GUIDE CARD if indicated.

9.3: ALTERED MENTAL STATE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Are there hallucinations? 2. Is there restlessness or agitation? 3. Is there an inability to focus or pay attention? 4. Are there Emotional disturbances? 5. Is the patient dazed or unalert? 6. Is there disorientation to time or place?	Simultaneous ALS/BLS • Fast pulse • Fever • Slow or rapid breathing • Cold or clammy skin • Dizziness or feeling faint • Uncontrolled shivering • Headache BLS Dispatch • Confusion with loss of consciousness • Confusion after a head injury • Confusion has come on suddenly in someone with diabetes. • Weakness on one side of the body • Slurred speech	1. General pre-arrival instructions/interventions 2. Stay with the person – tell them who you are and where they are, and keep reassuring them. 3. Do not stop the person moving around unless they are in danger. 4. Do not ask lots of questions while they are feeling confused. 5. Use simple words and short sentences.

9.4: ANIMAL BITES

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the animal contained? 2. What type of animal bit the patient? 3. Is the patient short of breath, or does breathing hurt? 4. What part of the body was bitten? 5. Is the patient bleeding?	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Decreased level of consciousness. • Uncontrolled bleeding after attempts to control. • Serious neck or face bites from animal attacks. • Bites from known venomous animals. BLS Dispatch • Controlled bleeding • Swelling at the bite site • Bite below the neck, non-poisonous	1. General pre-arrival instructions/interventions 2. Contain the animal, if possible. 3. Lock away any pets. 4. If severe bleeding, go to BLEEDING. 5. If there is little or no bleeding, irrigate human and animal bites with copious amounts of water. 6. Apply direct pressure to the wound. 7. Have the patient lie down, cover them with a blanket and try to calm them. 8. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY. 9. Do not elevate the extremity. 10. Do not use ice. 11. Do not attempt to remove the venom. 12. For jellyfish stings: Washing with seawater is often the best first practice here, alongside careful removal of tentacles still in situ.

9.5: ASSAULT - DOMESTIC, SEXUAL

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the assailant nearby? 2. Are you safe? 3. Was it a physical assault or a sexual assault? 4. How was the victim assaulted? (Stabbing, gunshot or major trauma go to the appropriate card) 5. What part of the patient is injured? 6. Is the patient bleeding? IF YES, go to BLEEDING	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Decreased level of consciousness. • Crushing injury (except to hands or feet) • Puncture injury (head, neck, torso, thigh) • Multiple extremity fractures. • Femur (thigh) fracture. • Uncontrolled bleeding. BLS Dispatch • Penetrating/crushing injury to hands or feet. • Isolated extremity fracture. • Minor injuries. • Unknown injuries. • Concerned caller without apparent injuries to the victim. • Police request standby/check for injuries.	1. General pre-arrival instructions/interventions 2. Remain in a safe place, away from the assailant. 3. Obtain a description of the assailant(s) 4. Advise the patient not to change clothing, bathe or shower. 5. Keep the patient warm. 6. Have the patient lie down, cover them with a blanket and try to calm them. 7. Do not touch weapons. 8. Do not allow the patient any food or drink. 9. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY.

9.6: BACK PAIN

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Has the patient felt dizzy or fainted? - Does the patient have any other medical or surgical history? - Is the patient's pain due to an injury or a recent fall? - Is the patient incontinent of urine or has urinary retention? - Is the patient wearing a Medic Alert tag? IF YES, what does it say? - Does the patient have Addison's Disease or adrenal insufficiency?	Simultaneous ALS/BLS - Decreased level of consciousness. - Cardiac disease or adrenal insufficiency. - Non-traumatic back pain with prior history of heart problems. - Back pain with fainting or near fainting, patient over 50 years - Non-traumatic back pain with prior history of Addison's disease. <i>Symptoms of an Addison or "Adrenal" crisis include:</i> - Severe vomiting and diarrhoea - Dehydration - Low blood pressure - Loss of consciousness If not treated, an Addison crisis can be fatal. BLS Dispatch - Flank pain/back (Kidney stone). - Back pain (non-traumatic). - Back pain unspecified. - Chronic back pain	General pre-arrival instructions/interventions - If the pain is due to an injury, tell the patient not to move unless hazards are present. - Have the patient rest in the most comfortable position. - Nothing to eat or drink.

9.7: BLEEDING

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Where is the bleeding from? - If the patient is female with vaginal bleeding, could she be pregnant? If YES, go to PREGNANCY/CHILDBIRTH. - Does she have pain in the abdomen? IF YES, consider ABDOMINAL PAIN. - How much blood can you see? - How long have they been bleeding? - Is blood squirting out? (Arterial bleeding) - Is the patient a haemophiliac (a bleeder)? - Has the patient recently travelled outside of the state/country? If YES, where (Check ALERTS)	Simultaneous ALS/BLS - Decreased level of consciousness. - Any arterial bleeding. - Bleeding with a history of Haemophilia. - Rectal bleeding with significant blood loss. - Vomiting blood or coffee ground material. - Bleeding from the mouth with difficulty breathing. - Bleeding from the neck, groin, or armpit with significant blood loss. - Vaginal bleeding if over 20 weeks pregnant, associated with lower abdominal pain or fainting. BLS Dispatch - Minor bleeding from any other area that can be controlled by direct pressure.	General pre-arrival instructions/interventions - If bleeding, use a clean cloth and apply pressure. - Elevate bleeding extremities, and if a Tourniquet is available, use it by following the instructions on its package. - If a tooth is bleeding, locate it and DO NOT touch its root. Place the cloth directly over the wound; do not remove it even if it is soaked. - If nosebleed, tell the patient to apply direct pressure by pinching the nose tightly between their index finger and thumb, sit forward and hold it until help arrives. - Attempt to spit out blood. Swallowing blood may make the patient nauseous. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils. - Have the patient lie down, cover them with a blanket and try to calm them. - Advise the patient not to move, eat or drink anything.

9.8: BREATHING PROBLEMS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the patient on asthma medication or has ever used them? 2. Is the patient able to speak in complete sentences? 3. Is the patient drooling or having a hard time swallowing? 4. What has changed about their breathing to prompt you to call? 5. Has the patient ever had this problem before? 6. How long has this been going on? 7. Does the patient have to sit up to breathe? 8. What was the patient doing just before they became short of breath? 9. Does the patient have any other medical or surgical history? 10. Does the patient have any allergies? 11. If sudden onset, has the patient been hospitalised recently for childbirth or a broken leg? 12. If female, does the patient take medication for birth control?	Simultaneous ALS/BLS • Any patient complaining of breathing or respiratory difficulty, examples of symptoms may include: • Difficulty breathing with chest pain. • Unable to speak in complete sentences. • History of Asthma or respiratory problems. • Inhaled substance. • Recent childbirth/broken leg/ hospitalisation (within 2-3 months). • Drooling/difficulty swallowing. • Tingling or numbness in extremities/ around the mouth, 35 or older. BLS Dispatch • Cold symptoms. • Stuffy nose/congestion. • Oxygen bottle is empty. • Patient assist. • Long-term, no change.	1. General pre-arrival instructions/interventions 2. Keep the patient calm. 3. The patient may be more comfortable sitting up. 4. Tell the patient not to exert themselves.

9.9: BURNS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- How was the patient burned? THERMAL – Is anything on the patient still burning? - IF YES, Stop the burning. - Place the burned area in cool water (not ice) if convenient. - Electrical – go to ELECTROCUTION. - What chemical caused the burn? - Where is the patient burned? - IF HEAD OR FACE: Is the patient short of breath, coughing, or does it hurt to breathe? - Are there burns around their mouth and nose? - Is the patient having difficulty swallowing? - Are there any other injuries?	Simultaneous ALS/BLS - Decreased level of consciousness. - Burns to the airway, nose, and mouth. - Hoarseness, difficulty talking or swallowing. - Burns over 20% of the body surface. - Electrical Burns/Electrocution from 220 volts or greater power lines/panel boxes. 2nd and 3rd-degree burns (partial or full thickness) - to palms (hands), soles (feet), groin BLS Dispatch - Less than 20% body surface burned. - Spilt hot liquids. - Chemical burns to the eyes. - Small burn from a match, cigarette. - Household electric shock. - Battery explosion. - Freezer burns.	General pre-arrival instructions/interventions THERMAL - Place the burned area in cool running water, if convenient. - Coach the caller on measuring TBSA (e.g. rule of palms) and quickly identify 1st, 2nd & 3rd degree burns. CHEMICAL - Have the patient remove contaminated clothing, if possible. - If chemical, get information on the chemical (MSDS Sheet if available). - If the chemical is powder, brush it off; do not use water. - Flush chemical burns from the eyes with water. - Remove contact lenses if present.

9.10: CARDIAC ARREST

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
If unsure about consciousness: - Does the patient respond to you? Talk to you? Answer questions? Hear you? - Does the patient move? Flinch? Move arms or legs? - Are the pupils fixed and dilated? If unsure about breathing: - Look and see if the chest rises and falls. Listen for the sound, frequency and description of breaths. - Agonal respirations are often reported as gasping, snoring, or gurgling, barely breathing, moaning, weak or heavy.	Simultaneous ALS/BLS - Unresponsive - Unconscious / not breathing adequately (Agonal) or not at all. - Possible cardiac arrest of unknown origin. - Delayed response. BLS Dispatch - Follow local protocol. - Confirmed hospice. - Expected death.	GO TO CPR CARD FOR THE APPROPRIATE AGE GROUP. - Age 8 years and above – ADULT CPR INSTRUCTIONS. - Age 1 year to 8 years - CHILD CPR INSTRUCTIONS. - Age 0 to 1 year – INFANT CPR INSTRUCTIONS. - CPR should be attempted with rescue breathing for an adult patient in cardiac arrest after asphyxia (unable to breathe), a drug overdose or drowning.

9.10.1: ADULT CPR INSTRUCTIONS (> 8 YEARS)

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Assess the person's surroundings for any danger and ensure that it is safe 2. Is the person seated or on the floor? 3. Call the person by name if you know them 4. Check the person's responsiveness by tapping their shoulder 5. Check if they are breathing 6. Check if they have a pulse 7. Ask the Rescuer if they are trained in Basic First Aid skills or BLS 8. If the person is breathing, place them in a recovery position 9. If not responsive, start CPR.	Simultaneous ALS/BLS • Unresponsive • Unconscious/not breathing adequately (Agonal) or not at all • Possible DOA (dead on arrival) of unknown origin • Delayed response BLS Dispatch • Confirmed hospice (terminally ill) • Expected death	1. CHECK the scene for safety, form an initial impression and use personal protective equipment (PPE) 2. If the person appears unresponsive, CHECK for responsiveness, breathing, life-threatening bleeding or other life-threatening conditions using shout-tap-shout 3. If the person does not respond and is not breathing or only gasping, call for HELP and an AED if available, or tell someone to do so HOW TO PERFORM CPR 1. Kneel beside the person. Place the person on their back on a firm, flat surface. 2. Give chest compressions • Hand position: Two hands centred on the chest • Body position: Shoulders directly over hands; elbows locked • Depth: At least 5 cm • Rate: 100 to 120 per minute • Allow the chest to return to its normal position after each compression • Minimise interruptions to chest compressions to less than 10 seconds • Use an AED as soon as one is available! • Continue giving chest compressions until EMS arrives.

9.10.2: CHILD CPR INSTRUCTIONS (1 TO 8 YEARS)

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Assess the child's surroundings for any danger and ensure that it is safe 2. Is the child seated or on the floor? 3. Call the child by name if you know them 4. Check the child's responsiveness by tapping their shoulder 5. Check if they are breathing 6. Check if they have a pulse 7. Ask the Rescuer if they are trained in Basic First Aid skills or BLS 8. If the child is breathing, place them in a recovery position 9. If not responsive, start CPR.	Simultaneous ALS/BLS • Unresponsive • Unconscious/not breathing adequately (Agonal) or not at all • Possible DOA (dead on arrival) of unknown origin • Delayed response BLS Dispatch • Confirmed hospice (terminally ill) • Expected death	1. CHECK the scene for safety, form an initial impression and use personal protective equipment (PPE) 2. If the child appears unresponsive, CHECK for responsiveness, breathing, life-threatening bleeding or other life-threatening conditions using shout-tap-shout 3. If the child does not respond and is not breathing or only gasping, call for HELP and an AED if available, or tell someone to do so HOW TO PERFORM CPR 1. Kneel beside the person. Place the child on their back on a firm, flat surface. 2. Give chest compressions • Hand position: Two hands centred on the chest • Body position: Shoulders directly over hands; elbows locked • Depth: At least 5 cm • Rate: 100 to 120 per minute • Allow the chest to return to its normal position after each compression • Minimise interruptions to chest compressions to less than 10 seconds • Use an AED as soon as one is available! • Continue giving chest compressions until EMS arrives.

9.10.3: INFANT CPR INSTRUCTIONS (0 – 1 YEAR)

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Assess the infant's surroundings for any danger and ensure that it is safe 2. Check the infant's responsiveness by gently tapping the feet 3. Check if the infant is breathing 4. Check if the infant has a branchial pulse 5. Ask the Rescuer if they are trained in Basic First Aid skills or BLS 6. If the infant is breathing, place them in the recovery position 7. If not responsive, start CPR for an infant	Simultaneous ALS/BLS • Unresponsive • Unconscious/not breathing adequately (Agonal) or not at all • Possible DOA (dead on arrival) of unknown origin BLS Dispatch • Expected death	1. CHECK the scene for safety, form an initial impression and use personal protective equipment (PPE). 2. If the infant appears unresponsive, CHECK for responsiveness. 3. If the infant does not respond and is not breathing or only gasping, call for HELP and an AED if available, or tell someone to do so. HOW TO PERFORM INFANT CPR 1. Place the baby on a firm surface and open their airway. To do this, place one hand on their forehead and gently tilt their head back. With your other hand, use your fingertip and gently lift the chin. 2. Put two fingers in the centre of the baby's chest and push down a third of the depth of the chest. Release the pressure, allowing the chest to come back up. Repeat this at a rate of 100 to 120 pumps per minute. 3. After 30 pumps, open the airway and give two breaths. Keep alternating 30 pumps with two puffs (30:2) until: • emergency help arrives and takes over • the baby starts showing signs of life and begins to breathe normally. 4. If the baby shows signs of becoming responsive, such as coughing, opening their eyes, making a noise, or starting to breathe normally, put them in the recovery position. 5. Monitor their level of response and prepare to give CPR again if necessary.

9.11: CHEST PAIN / HEART PROBLEMS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Where in the chest is the pain located? 2. Does the patient feel pain anywhere else? If so, where? 3. How long has the pain been present? 4. Is the patient sweating profusely? 5. Is the patient nauseated or vomiting? 6. Is the patient weak, dizzy, or faint? 7. How does the patient act when they sit up? 8. Does the pain change when the person breathes or moves? 9. Has the patient ever had a heart problem, heart surgery, a device to help their heart work or a previous heart attack? 10. Is the patient experiencing a rapid heart rate with chest pain?	Simultaneous ALS/BLS • Patient complaining of chest pain with any of the critical symptoms: Short of breath, nausea, diaphoretic (sweating profusely), rapid heart rate, syncope (weak, dizzy or faint) or cocaine/crack (drug) use. • Patient over 35 with any critical symptoms. • Decreased level of consciousness. BLS Dispatch • Patients under 35 without critical symptoms.	1. General pre-arrival instructions/interventions 2. Have the patient sit or lie down, whichever is more comfortable. 3. Loosen any tight clothing. 4. Keep the patient calm. 5. Can the patient take aspirin? IF YES, have they had any bleeding from the mouth or rectum? If there is no bleeding, advise the caller to assist the patient in taking one full-size (325mg) adult aspirin or four low-dose (81mg) tablets. Have the patient chew them before swallowing. 6. Does the patient have nitroglycerin? IF YES, has the patient taken one? If not taken, take as the physician has directed (patient should be seated).

9.12: CHOKING

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Are the victims' hands on their necks 2. No verbalisation? 3. No response? 4. No breathing?	Simultaneous ALS/BLS • Hands-on the neck, conscious and no verbalisation. BLS Dispatch • The patient is coughing but relentlessly.	1. General pre-arrival instructions/interventions 2. Encourage the victim to cough. 3. Give five back blows between the person's shoulder blades with the heel of your hand. 4. Give five abdominal thrusts just above the belly button. 5. Alternate between five blows and five abdominal thrusts until the foreign body comes out.

9.13: CORD PROLAPSE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the mother in labour? 2. Has a medical professional been notified? 3. Is the mother lying down with her legs elevated? 4. Has the mother or anyone else noticed a cord protruding from the vagina? 5. Is the cord pulsating or not pulsating? 6. Has the mother experienced a sudden increase or decrease in fetal movement? 7. Is the mother or baby showing any signs of distress, such as difficulty breathing or a change in skin colour? 8. Are there any previous histories, such as multiple births or a breech presentation?	Simultaneous ALS/BLS • Confirmed cord prolapse with foetal distress/lack of fetal movements • History of previous scar or bad obstetric history • Mother with strong contractions BLS Dispatch • Present foetal heart • Mother has had no contractions in the last 30 minutes • Mother with no difficulty in breathing • No bad obstetrics history previously	1. General pre-arrival instructions/interventions 2. Help the mother to lie down on her back with her head elevated and her hips raised slightly, either with pillows or by elevating the foot of the bed. 3. Avoid touching the cord, as this can cause further compression and reduce the blood flow to the baby. 4. Keep a close eye on the mother for any signs of distress, such as difficulty breathing or a change in skin colour. 5. Calm the mother and reassure her that help is on the way. Encourage her to take slow, deep breaths and to avoid pushing or straining. 6. Call back in case the mother's condition changes or if there are any changes in the baby's heart rate or other vital signs.

9.14: DIABETIC COMPLICATIONS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the patient on insulin? IF SO, when did they take their medication? 2. When did the patient last eat? 3. Does the patient have a glucose meter? IF YES, do you have a current level (Range usually between 70 and 180) 4. Is the patient acting in their usual manner? IF NOT, what is different? 5. Are they dizzy, weak, or feeling faint? 6. Is the patient complaining of any pain? Where is it located? 7. Is the patient sweating profusely? 8. Has the patient had a seizure?	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Decreased level of consciousness. • Unusual behaviour/acting strange. • Profuse sweating. • Seizure BLS Dispatch • Awake / Alert. • Not feeling well.	1. General pre-arrival instructions/interventions 2. Nothing by mouth if the patient is unable to take it by themselves 3. If the patient is conscious enough to swallow and the patient's blood glucose level is known and is below 3.5 mmol/L, or the blood glucose level is NOT KNOWN, and the patient is acting inappropriately, then give juice with 2 to 3 teaspoons of sugar in it. Providing this amount of sugar to a person with high blood glucose levels will not harm them and may benefit someone with low levels.

9.15: DROWNING

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Does the patient have a response? 2. Is the patient breathing?	Simultaneous ALS/BLS - No breathing and unconscious. - Unconscious and laboured breathing. BLS Dispatch - Conscious and breathing	1. GO TO CPR CARD FOR THE APPROPRIATE AGE GROUP. 2. Lie the patient in the recovery position if they are responsive 3. General pre-arrival instructions/interventions 4. Prevent hypothermia by covering the victim with a blanket.



9.16: ELECTRICAL INJURY

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Are there signs and symptoms of electrical injury? - Are there entry/exit wounds?	Simultaneous ALS/BLS - No breathing and unconscious. - Unconscious and laboured breathing. - Visible burns/injuries BLS Dispatch - Patient out of danger - Gained consciousness - No visible burns/injuries	General pre-arrival instructions/interventions - Beware of ground moisture. - Do not touch the patient if still in contact with the source of electricity. - Beware of liquid spills that may conduct electricity - If it is safe to do so, turn off the power. - Lock away any pets. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils.

9.17: EPISTAXIS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the patient still bleeding? Does the patient apply direct pressure for more than 20 minutes?	Simultaneous ALS/BLS - Still bleeding after applying direct pressure for 20 minutes BLS Dispatch - The patient is still bleeding, but did not apply direct pressure on the nostrils.	General pre-arrival instructions/interventions - Calm the patient and instruct them to avoid talking. - Ask the patient to blow their nose to remove clots and have the patient squeeze the distal alae continuously while sitting up, bent forward at the waist over a vomit bucket, and expectorating blood for 15 minutes. - Avoid the leaning backwards position. - Avoid swallowing the blood.

9.18: EXPLOSION

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Can you check yourself and others around you for injuries? 2. Can you check the area around you to determine if it is safe to leave or safer to stay? 3. Is it safe to evacuate? If so, do it as quickly and safely as possible, as secondary devices and structural damage may continue to occur. 4. Proceed to Rally Points. 5. If you are trapped in a structure, alert others to your situation in any way you can.	Simultaneous ALS/BLS • Establish the type of explosion and dispatch the appropriate ambulance • Appropriate and adequate PPs must be available • Activate the HAZMAT team if available	1. General pre-arrival instructions/interventions 2. Evacuate the building quickly and calmly. For example, be sure to identify more than one way to exit each building. If the nearest exit is blocked, you'll need another way out. 3. If things are falling off bookshelves or from the ceiling, take refuge under a sturdy table or desk. 4. If there is a fire, stay low to the floor and exit the building quickly. 5. If you are trapped in debris, tap on a pipe or wall so that rescuers can hear where you are. 6. Help others escape the building and move to your designated assembly area, a safe distance away. 7. Keep streets and walkways clear for emergency vehicles and responders. 8. Do not attempt to rescue people who are inside a collapsed or compromised building unless you have received specific training to do so. Wait for emergency personnel to arrive.

9.19: EYE INJURIES

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. What caused the injury? 2. Is the eyeball cut open or leaking fluid? 3. Are there any other injuries? IF YES, go to APPROPRIATE GUIDE CARD.	Simultaneous ALS/BLS • Unconscious / not breathing normally. • Decreased level of consciousness. • Uncontrolled bleeding. • Vomiting • Abnormal pupil sizes BLS Dispatch • Any eye injury.	1. General pre-arrival instructions/interventions 2. Advise the patient not to move. 3. Have the patient sit down. 4. Do not remove any penetrating objects. 5. If the eyeball is cut or injured, do not touch, irrigate, or bandage 6. If a chemical injury occurs, flush immediately with water. 7. Remove contact lenses. 8. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY. 9. Have the patient lie down, cover them with a blanket and try to calm them. 10. Nothing to eat or drink.

9.20: FALLS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- How far did the patient fall? - What kind of surface did they land on? - Did the patient lose consciousness? - Are there any obvious injuries? What are they? - Did the patient complain of pain or illness just before the fall? - Is the patient able to move their fingers and toes? (Do not have them move any other body part.) - Is the patient bleeding? IF YES, go to BLEEDING	Simultaneous ALS/BLS - Decreased level of consciousness. - Signs/symptoms of Shock. - Falls greater than 10 feet. - Falls associated with or preceded by pain, discomfort in the chest, dizziness, headache, or diabetes. - Patient paralysed. - Uncontrolled bleeding. - Multiple extremity fractures. - Femur (thigh) fracture. BLS Dispatch - Unconscious, but now conscious without critical symptoms. - Falls less than 10 feet. - Neck or back pain without critical symptoms. - Controlled bleeding. - Cuts, bumps, or bruises. - Isolated extremity fracture.	General pre-arrival instructions/interventions - Do not move the patient if there are no hazards. - Advise the patient not to move. - Have the patient lie down, cover them with a blanket and try to calm them. - No food or drink. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY.

9.21: FEBRILE CHILD

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- How old is the child? - Is there a bulging fontanel or neck stiffness? - Is there a history of malaria? - Is the child unable to eat or breastfeed? - Is there any seizure episode?	Simultaneous ALS/BLS - Age less than two months - Bulging fontanel or neck stiffness - History of malaria? - Febrile and unable to eat or breastfeed - Febrile child with a seizure BLS Dispatch - Febrile child and able to eat or breastfeed - Febrile child but not seizing.	General pre-arrival instructions/interventions - Apply a cold compress. - Administer antipyretics if available. Go to SEIZURES /CONVULSIONS. - Eat and breastfeed if the patient is conscious.

9.22: FRACTURES

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Do you have any bleeding from the site? 2. Do you have pain? 3. Is there more than one site of injury? 4. Do you have difficulty using the affected part? 5. Is there normal sensation below the affected part? 6. Is there a change of colour of the skin below the affected part?	Simultaneous ALS/BLS • Weakness of extremity with a history of falling injury • If severe pain • Multiple sites • Unable to use the affected part. • Severe bleeding. BLS Dispatch • Single-site forearm or lower extremity fracture and no difficulty in using the affected part.	1. General pre-arrival instructions/interventions 2. Make the alignment in a normal anatomical position. 3. Don't try to move the affected part (immobilise). 4. Apply splint (with available device) 5. Tie the affected part (limb) with the normal part, go to BLEEDING. 6. Provide analgesics if available & patient is conscious. 7. Reassure the patient 8. Link to a nearby facility.

9.23: HANGING / STRANGULATION

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Does the patient have a response? 2. Is the patient breathing?	Simultaneous ALS/BLS • No breathing and unconscious. • Unconscious and breathing. BLS Dispatch • Conscious and breathing	1. GO TO CPR CARD FOR THE APPROPRIATE AGE GROUP. 2. Lie the patient in the recovery position if they are responsive 3. General pre-arrival instructions/interventions 4. Reassure the patient

9.24: HEADACHE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Does the patient have a headache history? - Is the headache different from the headaches the patient has had in the past? - Did the headache come on suddenly or gradually? - What was the patient doing when the headache started? - How is the patient acting? If unusual, how? - Does the patient take blood thinners? - Does the patient know where they are and who they are? - Does the patient have pain anywhere else? IF YES, Where? - Has the patient had a recent illness, injury or trip to an Emergency Department? IF YES, for what? - Is the patient wearing a Medic Alert Tag? IF YES, what does it say?	Simultaneous ALS/BLS - Headache with these critical symptoms: decreased level of consciousness, mental status change, worst headache ever, sudden onset, visual disturbance, with no history of migraines. BLS Dispatch - Headache without critical symptoms.	General pre-arrival instructions/interventions - Nothing by mouth.

9.25: HEAT / COLD EXPOSURE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. What happened? 2. What was the source of the heat or cold? Heat-Related 3. Is the patient sweating profusely? 4. Is the patient confused, disoriented or acting strange? 5. Is the patient having hallucinations? 6. Is the patient dizzy, weak, or feeling faint? Cold-Related 1. Is the patient shivering or post-shivering 2. Is there obvious frostbite to the face/ extremities	Simultaneous ALS/BLS • Decreased level of consciousness. • High body temperature without sweating. • Confused/disoriented/hallucinations. • Fainting (Syncope). • Cold Water Submersion. • Narcotics and Psych Medications may exacerbate and/or mask symptoms. BLS Dispatch • Patient with uncontrollable shivering. • Heat Exhaustion: Nausea, vomiting, fatigue, headaches, muscle cramps, dizziness, with no critical symptoms.	1. General pre-arrival instructions/interventions 2. Remove from a hot/cold environment if possible. 3. If the patient is cold and dry, move to a warm environment and cover the patient. 4. If the patient is cold and wet, move to a warm place. 5. Remove any wet clothing. 6. If the patient is overheated, have them lie down in a cool environment, remove clothing and cover the patient. 7. Loosen clothing to assist in cooling. 8. Spray tepid water and fan the patient. 9. Consider a cold compress in the 'HULK' zones – Head/ Neck, Underarm/Axilla, Loin/Groin, Knees. 10. Nothing by mouth if heat stroke is indicated or there is a decreased level of consciousness. 11. Do not rub frostbitten extremities.

9.26: INDUSTRIAL ACCIDENTS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. What happened? If the patient is trapped in or under an object, what part of the person is trapped? 2. Are there any obvious injuries? What are they? 3. If amputation: What part of the body has been amputated? Do you have the amputated parts? 4. Is the patient bleeding? IF YES, go to BLEEDING. 5. If burned, go to BURNS. 6. If Electrocution: go to ELECTRICAL INJURY. 7. Is the patient able to move their fingers and toes? DO NOT have them move any other parts of their body.	Simultaneous ALS/BLS • Patient entrapped. PROMPT (Dispatch Rescue Unit) • Accident with crushing or penetrating head, neck, torso, or thigh injury. • Decreased level of consciousness. • Amputation other than fingers/toes. • Patient paralysed. • Uncontrolled bleeding. • Multiple extremity fractures. • Femur (thigh) fracture. BLS Dispatch • Unconscious, but now conscious without critical symptoms. • Amputation/entrapment of fingers/toes. • Neck or back pain without critical symptoms. • Controlled bleeding. • Cuts, bumps, or bruises. • Patient assist. • Involved in an accident, no complaints.	1. General pre-arrival instructions/interventions 2. If machinery is involved, turn it off (attempt to locate the maintenance person). 3. Nothing to eat or drink. 4. Do not move the patient if there are no hazards. 5. Locate any amputated parts and place them in a clean plastic bag, NOT ON ICE. 6. Advise the patient not to move. 7. If teeth are located, DO NOT touch the root; place them in milk or clean water. 8. Do not enter a confined space to tend to the patient. 9. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY. 10. Have the patient lie down, cover them with a blanket and try to calm them.

9.27: INFECTIOUS DISEASE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the patient complaining of fever, headache, tiredness (can be aroused), cough, sore throat, runny or stuffy nose, body aches, diarrhoea or vomiting (more common among children than adults)? 2. Is there any unusual bleeding from any part of the body? IF YES, Where? 3. Does the patient have a rash or blister on their body? IF YES: "Where?" " 4. Is the patient sensitive to light? Check the ALERT Card for current conditions 5. Check breathing: Is the patient short of breath or unable to speak in complete sentences? 6. Recent Travel: Has the patient travelled outside of their normal area within the last month? If so: "Where?"	Simultaneous ALS/BLS • Difficulty breathing • Uncontrolled bleeding • Decreased level of consciousness BLS Dispatch • No critical symptoms	1. General pre-arrival instructions/interventions 2. Do not touch the patient without using gloves. 3. Keep the patient in an isolated area. Don't allow the patient to move about. Prevent additional people from close contact. 4. Observe the level of consciousness and breathing 5. Keep the windows and doors of the room open to aid air circulation. 6. Cover the patient's mouth with a clean cloth or tissue when coughing. 7. Keep the materials used by the patient separately in a polythene bag for appropriate disposal. 8. Try to obtain the names of people who have been in close contact with the patient. If they are present, ask them to remain until emergency services arrive to get their information.

9.28: PERSISTENT VOMITING AND DIARRHEA IN CHILDREN

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1.How frequent is the vomiting and diarrhoea? 2.Is there any sunken eyeball? 3.Is the baby alert? 4.Is the baby eager to drink?	Simultaneous ALS/BLS - Persistent vomiting, diarrhoea and sunken eyeball or a decrease in urine output, not alert. (Severe Dehydration). - Frequent diarrhoea and vomiting & sunken eyeball, eager to drink. (Moderate Dehydration) BLS Dispatch - Less frequent diarrhoea, alert baby & no sunken eyeball. (Mild Dehydration)	1. General pre-arrival instructions/interventions 2. Give the baby ORS for each loss if they can drink. 3. Link to a nearby health facility.

9.29: POISONING – INGESTIONS / OVERDOSE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Do you have any idea what the patient took? Get the name of the product or substance. Contact Poison Control. Was it a prescription medication, nonprescription over-the-counter medication, herbal supplement, street drug or a combination of medications 2. Has the patient consumed alcohol? 3. If cocaine or crack, is the patient complaining of any pain? 4. Is the patient having difficulty swallowing? 5. Is the patient acting normally? IF NOT, what is different?	Simultaneous ALS/BLS Patient with these critical symptoms: • Unconscious/not breathing normally. • Any overdose of medication or Alcohol intoxication with an altered level of consciousness. • Combined alcohol and drug overdose. • Cocaine/crack with chest pain. • Ingestion of household cleaners, antifreeze, solvents, methanol, cyanide, and insecticides. • Difficulty swallowing. • Pregnancy or Obesity BLS Dispatch • Drugs, intentional/accidental ingestion without critical symptoms. • 3rd party report, caller not with the patient. • Reported overdose, patient denies taking medications, or it is unknown if medications/substances were taken. • Known alcohol intoxication, without other drugs, can be aroused.	1. General pre-arrival instructions/interventions 2. <i>If the substance can be identified as Heroin or other Opioid:</i> Do you have a NARCAN or NALOXONE kit? IF YES, have you used it as directed? If they have not used it, use it following the directions on the package. 3. Keep the patient in the area/house if safe. 4. Get the container of the substance taken if it is at the scene. 5. Do not force the patient to take any food or drinks or place the patient in the shower. 6. Nothing by mouth, including Ipecac, unless advised by Poison Control. 7. Monitor the patient's breathing and level of consciousness.

9.30: POISONING - INHALATION / CARBON MONOXIDE

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is a Carbon monoxide Detector activated? 2. Is the patient complaining of headache, confusion, weakness, fatigue, nausea, vomiting or dizziness? 3. Is the patient breathing normally? IF NO, go to BREATHING PROBLEMS. 4. What is the source of the inhaled substance? 5. If it is a commercial property, is the MSDS sheet available?	Simultaneous ALS/BLS • Carbon monoxide detector activation with Critical Symptoms: unconscious/ LOC/not breathing normally. • Decreased level of consciousness. • Inhalation of household cleaners, antifreeze, solvents, methanol, cyanide, or insecticides with difficulty swallowing/breathing BLS Dispatch • Chemicals on the patient's skin or clothing, no critical symptoms. • Third-party report, the caller is not with the patient.	1. General pre-arrival instructions/interventions 2. Get the patient to fresh air immediately. 3. If unable to go outside, open all doors and windows. 4. If the caller cannot move the patient or open windows, ask the caller to remain outside until help arrives. 5. Turn off any appliance with an open flame. (heaters, stoves, fireplaces, etc.)

9.31: PREGNANCY / CHILDBIRTH

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Frequency of contractions 2. Duration of contractions 3. Intensity of contractions 4. Rupture of the amniotic sac (time and presence of meconium) 5. Previous pregnancies and deliveries (complications, vaginal delivery, C-section) 6. Pre-existing medical conditions (HTN, DM, seizure, cardiac) 7. Medications taken before labour 8. Vaginal bleeding 9. Abdominal pain	Simultaneous ALS/BLS • Excessive vaginal bleeding • Low BP • Poor contractions • Lack of foetal heart rate • Pre-eclampsia • Cord prolapses • Breech presentation BLS Dispatch • Delivery not imminent • Vaginal bleeding without fainting <20 weeks pregnant • Abdominal injury <20 weeks pregnant • Water broke • Pregnant <20 weeks or menstrual with any of the following; cramps, pelvic pain, spotting.	1. General pre-arrival instructions/interventions 2. Do not use the toilet 3. Do not flush the toilet or dispose of any used pads. 4. Have the patient lie down on their left side 5. Lock away any pets

9.32: PSYCHIATRIC / BEHAVIORAL PROBLEMS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- If the caller knows the patient, is the patient acting in their usual manner? IF NOT, what is different or unusual? - Is the patient: - Acting violently, aggressively, shouting or yelling? - Removing their clothing or naked? - Sweating profusely? - Breathing rapidly or drooling? (Excited Delirium) - Is the patient a diabetic? Consider DIABETIC COMPLICATIONS. - Does the patient have a history of depression? - Does the patient have a history of harming themselves or others? - Has the patient ever attempted suicide? - Has the patient recently travelled outside of the county or country? IF YES, Where? (Check ALERTS).	Simultaneous ALS/BLS - Decreased level of consciousness. - Patient presenting with; - extreme violent or aggressive behaviour, - Sweating profusely - Removing clothes or naked - Rapid breathing, drooling - Incoherent shouting or yelling BLS Dispatch - Lacerated Wrist(s) with controlled bleeding. - Unusual behaviour with Psychiatric history. - Known alcohol intoxication without other drugs (can be aroused). - Threats against self or others. - Police request for standby - Patient is out of Psychiatric medications.	General pre-arrival instructions/interventions - Keep the patient in the area if safe. - Keep the patient calm, if possible. - If you feel you are in danger, leave the scene. - If suicide is indicated, try to determine the means. Attempt to help the patient using the appropriate Guide card—alert responders to hazards such as gas, chemicals, and weapons. - Suicidal callers may be reluctant to give their location. Use interrogation skills, the Automatic Location Identification screen, Phase II wireless information, and contacting the telephone service provider. Phase II allows call takers to receive the caller's wireless phone number and location information.

9.33: ROAD TRAFFIC CRASH

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Did you stop or drive by? 2. What type of vehicle(s) are involved? 3. How many patients are injured? 4. Are all of the patients free of the vehicle? 5. Is anyone trapped in the vehicle? 6. Was anyone thrown from the vehicle? 7. Are there any hazards present? (Is the scene safe?) Is there: Fire, Fluids leaking? (Consider HAZMAT), Wires down? 8. Describe what happened. Did the airbags deploy? How fast was the vehicle moving? 9. As injuries or medical conditions become known, go to the appropriate Guide card(s).	Simultaneous ALS/BLS Reported injuries with the following mechanisms: • Vehicle vs immovable objects. • Vehicles involved in a head-on or T-bone collision. • Car vs pedestrian, motorcycle or bicycle. • Patient(s) trapped or ejected. • Vehicle rollover. • Critical criteria: injuries to the head, neck, torso, or thigh. • Multiple Casualty Incident BLS Dispatch • Accident with injury, no critical criteria. • Police request standby/check for injuries	1. General pre-arrival instructions/interventions 2. Do not approach the vehicle if there is any indication of fire, downed wires or other hazards. 3. If able to enter the crash scene, DO NOT move patient(s) unless there are hazards. 4. Ensure the engine is switched off or is not running 5. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY.

9.34: SEIZURES / CONVULSIONS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is the patient still seizing? IF YES, how long has the patient been seizing? 2. Has the patient had a seizure before? 3. Is the patient on medication, or are they recreational drug users? 4. Has the patient had a recent head injury? 5. Is the patient a diabetic? Consider DIABETIC PROBLEMS. 6. IF PATIENT IS A CHILD, Has the child been sick? Does the child have a fever or feel hot? 7. IF PATIENT IS FEMALE: Is the woman pregnant? 8. Does the patient have a Medic Alert bracelet on? IF YES, what does it say?	Simultaneous ALS/BLS • Decreased level of consciousness. • Not breathing after the seizure stops. • Extended seizures greater than 5 minutes. • Multiple seizures. • Febrile seizures. • First-time seizure or seizure, unknown history. • Secondary to drug overdose, diabetes, pregnancy, or recent head injury. • Any seizure that is different from normal. BLS Dispatch • Single seizure with a history of seizure disorder.	1. General pre-arrival instructions/interventions 2. Clear the area around the patient. 3. Do not restrain the patient. 4. Do not place anything in the patient's mouth. 5. If the patient is a child, remove clothing to cool the patient if hot. 6. After the seizure has stopped, check to see if the patient is breathing. IF NO, GO TO CPR CARD FOR THE APPROPRIATE AGE GROUP. IF YES, have the patient lie on the side. 7. Monitor fever and breathing.

9.35: SPRAINS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Do you have pain? 2. Where is the site of pain? 3. Is there any swelling at the site? 4. Is there any misalignment? 5. Do you have difficulty using the affected part? 6. Is there normal sensation below the affected part? 7. Is there a change of colour of the skin below the affected part?	Simultaneous ALS/BLS • If there is pain and swelling. • If there is misalignment. BLS Dispatch • If there is pain only.	1. General pre-arrival instructions/interventions 2. Rest the affected part, 3. Immobilise the affected part. 4. Apply ice, elevate your limb 5. Give analgesics if available, go to FRACTURES 6. Rest the affected part 7. Apply ice. 8. If there is no response, go to the nearby health facility.

9.36: STABBING / GUNSHOT / ASSAULT

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. What part(s) of the body is injured? 2. When did this happen? 3. Was it intentional or an accident? If intentional, is the assailant still present? 4. What type of weapon was used? 5. Is the weapon still present? 6. Is more than one person injured? 8. Is there bleeding? IF YES, go to BLEEDING.	Simultaneous ALS/BLS • Unconscious / not breathing normally. • Decreased level of consciousness. • Uncontrolled bleeding. • Leg injury above the knee. • Wounds to the head, neck, torso, or thigh. • Multiple casualty incidents. BLS Dispatch • Wounds to the arms below the elbow or on the leg below the knee.	1. General pre-arrival instructions/interventions 2. Tell the caller to remain safe (beware of the assailant) 3. Do not disturb the scene or move weapons. 4. Do not pull out any penetrating weapons. 5. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils, go to TRAUMATIC INJURY. 6. Have the patient lie down, cover them with a blanket and try to calm them.

9.37: STROKE /CVA

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. When did this start? Does the patient have: 2. Sudden numbness or weakness of the face, arm or leg? (Especially on one side of the body). 3. Sudden confusion, trouble speaking (slurring) or understanding? 4. Sudden trouble seeing in one or both eyes? 5. Sudden trouble walking, dizziness, loss of balance or coordination? 6. Sudden severe headache? 7. Has the patient ever had a stroke? 8. Has the patient had any recent injury/trauma? 9. A history of diabetes? 10. Any other medical or surgical history?	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Marked change in the level of consciousness. • New onset of one-sided weakness with paralysis, facial droop, slurred speech, confusion, loss of vision, loss of coordination, severe headache BLS Dispatch • history of stroke (CVA) with no new changes.	1. General pre-arrival instructions/interventions 2. Keep the patient calm. 3. Don't allow the patient to move around. 4. If unconscious or having difficulty breathing, go to UNCONSCIOUS AIRWAY CONTROL. 5. Nothing by mouth (to eat or drink).

9.38: TRAUMATIC INJURY

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- How was the patient injured? - Where is the patient injured? - Describe what happened. Is the patient bleeding? If YES, go to BLEEDING. Indications of SHOCK: - Is the patient's skin cool, clammy, mottled, or profusely sweating? - Is the patient's breathing rapid and shallow? - Are the patient's pupils dilated, unequal or non-PEARL? - Does the patient appear confused? - Does the patient feel weak or tired? - Is the patient's mouth dry, or do they feel thirsty?	Simultaneous ALS/BLS - Unconscious / not breathing normally. - Decreased level of consciousness. - Penetrating/crushing injury to head, neck, torso, or thigh. - Multiple extremity fractures. - Leg injury above the knee. - Uncontrolled bleeding. - Indications of Shock. BLS Dispatch - Penetrating/crushing injury to hands or feet. Unknown or internal injuries without indication of Shock. - Minor injuries. - Concerned caller without apparent injuries to the victim. - Isolated extremity fracture. - Police request standby/check for injuries.	General pre-arrival instructions/interventions - Do not move the patient unless there are hazards to the patient. - Do not remove or touch impaled objects. - Do not disturb anything. Monitor for SHOCK: skin cool and clammy or mottled, rapid shallow breathing, fatigue, altered mental state, dilated pupils. - If teeth are located, DO NOT touch the root; place them in milk or clean water. Use care not to obstruct the airway or breathing. - Have the patient lie down, cover them with a blanket and try to calm them. - Locate any amputated parts and place them in a clean plastic bag, NOT ON ICE.

9.39: UNKNOWN ILLNESS

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
- Does the patient feel pain anywhere? If so, where? (Consider appropriate card; Back, Chest, Abdominal Pain) - Does the patient feel lightheaded or dizzy? - What is the patient complaining of? - How does the patient look? - Have you checked for a medic alert tag? If there is an alert tag, what does it say? - Is there insulin in the refrigerator? - Has the patient recently travelled outside of the town or country? IF YES: Where (Check ALERTS) - Does the patient have Addison's Disease or any other medical or surgical history? - Flu Symptoms – Is the patient complaining of fever, headache, tiredness (can be aroused), cough, sore throat, runny or stuffy nose, body aches or diarrhoea and vomiting (more common among children than adults)? For a more detailed interrogation, go to INFECTIOUS DISEASE.	Simultaneous ALS/BLS - Decreased level of consciousness. - Prior history of Addison's disease or adrenal insufficiency with dehydration, severe vomiting and diarrhoea or low blood pressure. Symptoms of an Addison or "adrenal" crisis include: - Severe vomiting and diarrhoea. - Dehydration. - Low blood pressure. - Loss of consciousness. If not treated, an Addison crisis can be fatal - Multiple fainting episodes. BLS Dispatch - Generalised weakness. - Medic alert from the alarm company. - Flu symptoms: (without critical signs or symptoms) - Other medical options, e.g. High blood pressure without critical symptoms. - High temperature. - Patient assist.	General pre-arrival instructions/interventions

9.40: UNKNOWN PERSON DOWN

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is there an obvious injury? 2. Can you see blood or any other fluid around the patient? 3. Have you checked for a medic alert tag? IF YES, what does it say? 4. Has the patient recently travelled outside of the town or country? IF YES, where? (Check ALERTS) 5. Does the patient have Addison's Disease or any other medical or surgical history?	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Decreased level of consciousness. • Multiple Casualty Incident BLS Dispatch • Unknown (Third Party Call) without indications of unconsciousness. • Patient talking, moving, sitting, or standing.	1. General pre-arrival instructions/interventions 2. If there is no danger, go to the patient to see if the patient is awake, breathing normally, or moving at all.

9.41: UNRESPONSIVE PATIENT

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
If the caller knows the patient: 1. Has the patient recently travelled outside of the country? IF YES: Where? (Check ALERTS) 2. Does the patient have any medical or surgical history?	Simultaneous ALS/BLS • Unconscious/not breathing normally. • Decreased level of consciousness. • Multiple Casualty Incident Criteria. BLS Dispatch • Unknown (Third Party Call) without indications of unconsciousness. • Patient talking, moving, sitting, or standing.	1. If there is no danger, go to the patient to see if the patient is awake, breathing normally, or moving at all. 2. If unconscious and NOT breathing normally, GO TO CPR CARD FOR THE APPROPRIATE AGE GROUP. 3. Lie the patient in the recovery position if they are responsive. 4. General pre-arrival instructions/interventions

9.42: VAGINAL BLEEDING

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is she term pregnant or early pregnancy? Parity? Gravida? 2. Is the bleeding massive? 3. Is she confused? 4. Does she have active bleeding? 5. Did she give birth recently?	Simultaneous ALS • Severe bleeding after birth. • Severe bleeding at term. • Severe vaginal bleeding with confusion BLS Dispatch • Vaginal bleeding, which is of sudden onset in a non-pregnant patient • Patient has not changed more than two pads in the last six hours • No intensive abdominal pain • No signs of fatigue or fainting	1. General pre-arrival instructions/interventions 2. Apply pressure to the lower abdomen and massage it. 3. Calm the patient and advise them to minimise movement. 4. If unconscious and breathing, put in the recovery position

9.43: WEAKNESS OF EXTREMITIES

KEY QUESTIONS	DISPATCH	PRE-ARRIVAL INSTRUCTIONS/INTERVENTIONS
1. Is there a history of fall injury? 2. Is there a history of LOC? 3. Think FAST: Is there a unilateral Facial droop Is there Arm drift? Is there any Speech difficulty? What was the Time of onset? 4. Is there a history of hypertension?	Simultaneous ALS/BLS • Weakness of extremity with facial drop and arm drift • Weakness of extremity with a history of fall injury BLS Dispatch • Pre-existing weakness, under medical care and in need of a medical escort • Conscious patient with no major complaint	1. General pre-arrival instructions/interventions 2. Protect the patient from falling. 3. Ask if the patient has any medical conditions that may be contributing to the weakness. 4. Put the patient in a recovery position 5. If there is bleeding, go to BLEEDING 6. If there is an associated fracture, go to FRACTURES

This image shows a blank sheet of white paper with horizontal ruling lines. The lines are evenly spaced and extend across the width of the page. There is a faint, light blue vertical band running down the center of the page, which appears to be a shadow or a design element. The overall appearance is that of a clean, unused piece of stationery.

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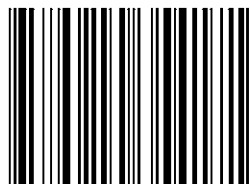
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